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Honorable Mehmet Oz
Administrator
Centers for Medicare & Medicaid Services (CMS)
Department of Health and Human Services
7500 Security Boulevard
Baltimore, Maryland 21244-1850

Honorable Thomas Keane
Assistant Secretary for Technology Policy
National Coordinator for Health Information Technology
Office of the Assistant Secretary for Technology Policy/Office of the National Coordinator for Health Information Technology (ASTP/ONC)
Department of Health and Human Services
330 C St SW
Washington, DC 20201

Re: *Request for Information; Health Technology Ecosystem*

Dear Administrator Oz and Assistant Secretary Keane,

On behalf of OCHIN, we thank you for the opportunity respond to the Centers for Medicare & Medicaid Services (CMS) and Assistant Secretary for Technology Policy (ASTP)/Office of the National Coordinator for Health Information Technology (ONC) request for information (RFI) on the health technology ecosystem. Building a modern health technology ecosystem will require an approach grounded in trust, practicality, and long-term sustainability. We **strongly support** expanding access to digital tools that empower patients and improve care while ensuring that such advances do not risk excluding patients and overburdening providers. To serve the full range of care delivery in America, the nation must invest in a digital infrastructure that includes rural and low-resourced communities where Medicare beneficiaries and older adults make up a substantial share of patients. This means embracing innovation, thinking outside the box, foundationally modernizing health IT systems, and rapidly and continually upskilling both providers and patients on the effective and safe use of new transformative tools.

OCHIN: Driving Innovation, Cost-Efficiency, And Self-Sufficiency

OCHIN is a **national nonprofit health information technology and research network** that includes over 2,000 community health care sites with 34,000 providers in 43 states – part of the backbone of the nation's public health and safety net infrastructure. With 25 years of experience, we know investing in cutting-edge technology empowers providers to offer higher-quality, data-driven care. We provide technology solutions, informatics, evidence-based research, and policy insights for rural and frontier health clinics, critical access hospitals (CAHs), and federally qualified health centers (FQHCs).

Building a Resilient Digital Health Ecosystem for Providers in Rural and Low-Resourced Areas

We offer the following overarching recommendations to guide the Administration's next phase of digital health transformation initiatives. Additional details and recommendations in the Appendix.

- **Invest in modern, certified health IT systems for providers in rural and low-resourced communities.** Far too many providers, disproportionately providers in rural communities, have not had sufficient resources to upgrade their antiquated IT systems – with many acquired well over a decade ago. We cannot achieve the goals of a health technology ecosystem unless all providers have access to up-to-date systems and technical support, which includes simplifying funding programs and sustaining long-term health IT capacity. Providers are eager to participate in digital innovation—but they need practical tools, sustained federal investment, and policies that reflect the realities of care delivery on the ground.
- **Advance TEFCA as the trusted national infrastructure.** Rapid, seamless, secure, accurate, and frictionless exchange of patient data is vital to improving patient care and reducing cost. We strongly support TEFCA's continued development as the primary means for secure exchange across use cases (e.g., treatment, public health, payment). We urge CMS and ASTP/ONC to coordinate federal participation and signal TEFCA as the preferred national exchange infrastructure – this includes reducing barriers to participation, including cost and onboarding complexity.
- **Pursue a patient-centered, accountable approach to identity and access.** A unique patient identifier (UPI) offers the most practical and privacy-conscious solution to accurate patient matching than third-party identity credentials. UPIs reduce the cybersecurity, workflow, and access risks as compared to credentialing systems that rely on external verification without accountability for errors. Further, such options do not account for the digital proficiency of many patient populations segments such as older adults. We urge you to prioritize a nationally coordinated UPI strategy – aligned with TEFCA – over models that could fragment identity, exclude hard-to-reach populations, or burden providers with verification tasks.
- **Ensure all policies reflect the digital readiness of providers and patients.** Many rural and low-resourced communities face ongoing barriers such as limited broadband, outdated systems, and low digital literacy. CMS and ASTP/ONC account for these gaps and prioritize practical support, including infrastructure investment, digital skilling, and flexible technology expectations.

CONCLUSION

We urge CMS and ASTP/ONC to pair its innovation efforts with real, ongoing support for the communities that often face the greatest challenges—and have the most to gain. We look forward to working with you to build a digital health ecosystem that serves the full spectrum of care delivery in America.

Please contact me at **stollj@ochin.org** if we can provide any additional information to support your efforts.

Sincerely,

Jennifer Stoll
Chief External Affairs Officer

APPENDIX

INTEROPERABILITY AND TEFCA

- **TEFCA must be the national backbone for health data exchange.** *(TD-6, PC-10, PR-6)*
  - We support expanding TEFCA across treatment, public health, payment, and public program administration—but its governance must prioritize providers, not just vendors.
  - TEFCA's structure should reflect that providers are custodians of patient data and legally accountable for privacy compliance.
- **Strengthen TEFCA's governance, audit, and enforcement roles.** *(PC-10, PR-6)*
  - Provider representation must be increased in oversight.
  - CMS and ASTP/ONC should invest in monitoring tools to prevent mischaracterized transactions (e.g., non-treatment data marked as treatment) and support fair, transparent dispute resolution processes.
- **Phase in new TEFCA use cases responsibly.** *(PC-10c)*
  - While treatment should remain the top priority, TEFCA must expand to support Public Health, Payment and Operations, Individual Access, and Government Benefits.
  - New TEFCA use cases should be optional first, then recommended, and finally required, allowing time for standards, funding, and workflows to mature.
  - Many safety-net providers interact with public programs like Medicaid, SSA disability eligibility determinations, and VA benefits for Title 38 veterans.
  - TEFCA could streamline verification, eligibility checks, and reporting if policy frameworks are aligned.
- **Eliminate financial and onboarding barriers to TEFCA participation.** *(PC-10, PR-6)*
  - Many providers in rural and low-resourced communities cannot afford legal review, technical integration, or administrative overhead.
  - CMS should offer technical assistance and financial incentives or bonus payments to join TEFCA.

INFORMATION BLOCKING

- **Expand the TEFCA Manner Exception.** *(PR-12)*
  - ASTP should expand the TEFCA Manner Exception so providers who use TEFCA to respond to requests are protected from information blocking penalties even if the requestor is not a TEFCA participant.
  - This would create a reliable compliance pathway and encourage wider adoption.
- **Shift enforcement toward clarity and usability.** *(PR-12, PC-13, PR-14)*
  - Complaint volume alone is not an indicator of effective implementation.
  - CMS and ASTP/ONC should invest in model workflows and guidance to help providers meet legal obligations confidently.

DIGITAL IDENTITY & CREDENTIALING

- **Support a national strategy for a unique patient identifier (UPI).** *(PC-14, PR-9, PR-10)*
  - A UPI offers a consistent, system-wide solution for patient matching, reduces duplication, and simplifies cross-system exchange.

    - Unlike digital identity credentials, UPIs can be designed specifically for healthcare and remain consistent across a person's lifetime and all care settings.
- **Digital identity credentials pose practical and structural challenges.** *(PC-14, PR-10)*
    - While digital credentials may enhance login security or access control, they depend on third-party verification processes that introduce privacy risks, increase provider burden, and exclude patients such as those who are not as digitally proficient.
    - These credentials are not inherently linked to clinical records and cannot replace a robust, healthcare-specific identifier.
    - Specifically, there can be multiple individuals with the same first and last name in a single household and there can be hundreds of individuals with the same first and in a region, including individuals with the same date of birth.
    - Incorrect identification with third parties misaligns accountability and liability as providers will face exposure when a third-party identification is not correct.
- **Protect the sanctity of the patient-provider relationship.** *(PC-14e, PR-9b)*
    - Identity workflows must include explicit, provider-mediated consent, especially when sensitive health data (e.g., behavioral, reproductive, or adolescent care) is involved.
    - Identity verification alone must not trigger automated data release without informed consent.
- **Align legal liability with control.** *(PC-14e, PR-10e)*
    - If data is disclosed via external platforms or automated tools, the entity responsible for operation execution should bear appropriate accountability.
    - For example, if a third-party credential servicing provider (CSP) asserts a patient's identity incorrectly, it should assume liability and the provider should be protected from any breach situation resulting from relying on that credential.
- **Patient matching must be durable over time and across systems.** *(PC-14, PR-10)*
    - According to the U.S. Census Bureau, a person in the U.S. is estimated to move 11.7 times in their lifetime.[1]
    - That means, on average, a patient could change their identifying information more than 11 times—multiplying the risk of errors, mismatches, and lost clinical context unless a consistent UPI is in place.
- **Privacy and patient safety are also at stake.** *(PC-14c, PR-10c)*
    - Today's fragmented identity processes require individuals to repeatedly share large amounts of personally identifiable information with every provider they visit, increasing the risk of both exposure and errors.
    - More concerning, poor identity resolution can result in overlays – when records from multiple patients are merged – causing a patient to have access to another patient's health information, which could result in an unauthorized disclosure under the Health Insurance Portability and Accountability Act (HIPAA), or even worse, a patient receiving treatment for another patient's disease.
- **Identity verification must not limit care access.** *(PC-14a, PC-14f, PR-9a)*

[1] https://www.census.gov/topics/population/migration/guidance/calculating-migration-expectancy.html

    - Credentialing systems that require specific devices, ID documents, or internet access will disproportionately impact rural and older adult populations.
    - Verification options should include in-person onboarding, caregiver proxies, and provider-facilitated consent processes.
    - Digital identity infrastructure must ensure patients without digital identity credentials (e.g., CLEAR, Login.gov, ID.me, other NIST 800-63-3 IAL2/AAL2 credentialing service providers (CSP)) access can still receive care.
- **Require increased oversight and reporting.** *(PC-14e, PR-9b)*
    - Credential servicing provider (CSP) will be high-value breach targets. CMS should require public breach reporting, transparency of false-positive rates, and audit trails.
    - Patients should also be able to revoke credentials at any time.
    - Providers must be able to override credentials when safety or legal concerns arise.

PROVIDER DIRECTORY MODERNIZATION

- **Consolidate—not duplicate—provider directory requirements**. *(TD-5, PA-4)*
    - Providers today must report the same information to PECOS, NPPES, payers, state boards, and commercial directories – often manually.
    - CMS should lead the development of a centralized, federated provider directory that replaces - not duplicates - existing directory requirements.
- **Build the directory on federated infrastructure.** *(TD-5)*
    - TEFCA provides a secure, interoperable foundation.
    - A directory should support machine-readable, API-based updates and rely on clear sources of truth:
        - CMS for National Provider Identity (NPI) and enrollment
        - Providers for contact, location, and specialty data
        - Payers for network status
        - Each entity for its own FHIR endpoints
- **Ensure strong data governance.** *(TD-5)*
    - Any national directory must include audit capabilities, attestation requirements, and transparent update mechanisms—not flat-file or manual processes.

DIGITAL QUALITY MEASUREMENT

- **Avoid penalizing rural and small practices during eCQM transition.** *(PR-8a, VB-1)*
    - Many providers in low-resourced settings still operate on legacy electronic health records (EHRs) that cannot generate QRDA files or FHIR requests.
    - CMS should:
        - Allow exclusions when ACOs meet overall patient reporting thresholds.
        - Invest in technical assistance and standards adoption (e.g., Bulk FHIR).
        - Align measure calculation responsibilities to CMS while enabling provider-led validation.
- **Support long-term interoperability for performance reporting.** *(PR-8a, VB-1)*

    - Any future standard should enable re-use of data for both care delivery and reporting, reducing duplication and allowing APMs to succeed without forcing provider dropout.

DATA EXCHANGE: PUSH-BASED AND PULL-BASED MODELS

- **Providers need continued support for event-driven (push-based) data exchange.** *(TD-16, PR-7)*
    - There are still many digital care coordination tools that depend on real-time (push) data (electronic referrals, discharge notifications, and provider-to-provider communication) instead of just pull-based exchange – so for providers to fully leverage digital health products - guidance should encompass support for event-driven standards
- **Pull-based only exchange is insufficient for real-world workflows.** *(TD-16)*
    - Clinical care often requires proactive alerts and updates.
    - Limiting the ecosystem to pull-based models may delay critical decisions and reduce the impact of digital tools at the point of care.
    - Push-based data exchange is essential for situation awareness.
- **Technology vendors and networks must support both models.** *(TD-16)*
    - Push-based exchange (e.g., Direct Secure Messaging, event notifications) should be placed on equal footing in federal guidance and certification criteria.
    - These tools are foundational for clinical coordination (for example, automated notification systems) and must not be sidelined in favor of pull-only standards.
- **Public health reporting depends on push-based mechanisms.** *(TD-16, PC-10c)*
    - If CMS and ONC encourage an ecosystem shift to pull-only, it would undermine established public health infrastructure.
    - Both exchange models are essential for a complete, responsive interoperability framework.